



معسكر الورقة الثانية للأطفال — الأسئلة المقالية



Essay Booklet – Day 1 (مقالي اليوم الأول)



Student Copy

Total Questions: 40 | Second Paper
Camp

BOOKLET TYPE

Essay Questions (مقالي)

TOTAL QUESTIONS

40 Questions

ACTIVE CAMP

Second Paper (الورقة الثانية)

Topic 1: Bronchial Asthma (Triggers, Investigations, Clinical Picture)

Question 1 Define Bronchial Asthma.

Question 2 Enumerate four risk factors associated with the epidemiology of childhood asthma.

Question 3 List four major pathophysiology changes responsible for airway obstruction in asthma.

Question 4 Enumerate four indoor or seasonal allergens that act as asthma triggers.

Question 5 Mention three common comorbid conditions associated with asthma that make it difficult to treat.

Question 6 List four key clinical signs and symptoms of an acute asthma episode.

Question 7 A 14-year-old boy presents with a history of cough and occasional wheezes at night, increasing during football matches. Enumerate the comprehensive Laboratory Investigations useful for this condition.

Question 8 List four life-threatening clinical signs found in a child with Status Asthmaticus.

Topic 2: Pneumonia

Question 9 Define Community-acquired pneumonia (CAP) and Hospital-acquired pneumonia (HAP).

Question 10 Mention the WHO definition of fast breathing (tachypnea) according to age groups.

Question 11 List four signs of respiratory distress that may occur in a child with pneumonia.

Question 12 Enumerate four local lung examination findings over the affected lung field in a pneumonia patient.

Question 13 List four non-respiratory gastrointestinal manifestations that can occur with pneumonia.

Question 14

Enumerate five clinical criteria that should lead to consideration of hospitalization/ICU admission for a child with Community-Acquired Pneumonia.

Question 15

Mention four diagnostic chest X-ray findings or clinical rules regarding the use of X-ray in pneumonia.

Question 16

Enumerate four causes for pneumonia that failed to resolve (Unresolved pneumonia) after antibiotic treatment, and enumerate two investigations that can be performed.

Question 17

List four thoracic cavity complications caused by the direct spread of pneumonia infection.

Question 18

Question 19

Question 20

Q1

Section II: Shock

Q2 Define Shock in pediatric patients.

Q3 List the clinical grading of Shock including the presentation of each grade.

Q4 Short Essay: Enumerate the main types of shock and detail the specific causes for each type as per the textbook.

Section III: Coma & Glasgow Coma Scale (GCS)

Q5 Define Coma and outline its primary and secondary brain lesion causes.

Q6 Mention the clinical grading system of Coma.

Q7 Detail the components and score allocation of the Pediatric Glasgow Coma Scale (GCS).

Q8 Mention the advantages of utilizing the Glasgow Coma Scale (GCS) in pediatric emergencies.

Title 1: Principles of family medicine

Question 1 Enumerate the four key elements of "Continuity of Care" as a core principle in family medicine demonstrated in managing this patient over his life course.

 Case Scenario:

Case 1: A 45-year-old male patient who has been registered with the same family physician for 10 years presents for a routine check-up. The physician manages his Hypertension, screens him for Colon Cancer, evaluates his psychosocial stress at work, and coordinates his care with a cardiologist when needed.

Question 2 List the five core services or dimensions integrated within the comprehensive and holistic approach of family medicine.

Title 2: Family physician- family health team- family health model

Question 3 Enumerate the five core parameters of the family physician's role as a "Five-Star Doctor" summarized by the World Health Organization (WHO).

 Case Scenario:

Case 2: A family physician is reviewing the monthly performance of the Family Health Center (FHC). He notices a conflict in the delegation of duties between the clinical nurse and the social worker, as well as a lack of proper documentation in the health records.

Question 4 Enumerate four specific members who belong strictly to the "Administrative and Support Staff" category of the Family Health Team.

Title 3: Basic benefit package

Question 5 Enumerate four essential laboratory investigations that are fully available at the level of the Family Health Unit (FHU) according to the BBP laboratory services guidelines.

 Case Scenario:

Case 3: A mother brings her infant to the clinic. The family physician is reviewing the components of the Basic Benefit Package (BBP) to verify which child clinical services and laboratory tests can be fully completed at the primary care facility.

Question 6 List the three sequential levels of service delivery through which the Basic Benefit Package is implemented within the family health delivery model.

Title 4: family - Family dynamics

Question 7 Enumerate the five main functional parameters of the family that represent its baseline societal obligations using the CAPRS framework.

 Case Scenario:

Case 4: A 14-year-old adolescent is brought to the clinic. Genogram analysis reveals a strong family history of early-onset coronary artery disease. Recently, the family experienced a major crisis due to the sudden death of the father, causing profound emotional instability.

Question 8 List four chronic or metabolic diseases in which a detailed family history contributes directly to calculating the patient's individual risk database.

Title 5: patient education

Question 9 Enumerate four fundamental reasons/clinical advantages that outline the core importance of implementing effective patient education in family medicine.

 Case Scenario:

Case 5: A 50-year-old male patient with newly diagnosed asymptomatic Hypertension tells the family physician, "I don't need to take my anti-hypertensive drugs daily; I will only take them on the days when I feel a headache".

Question 10 List four essential behavioral or structural principles that must guide the delivery of high-quality patient education during a clinical consultation.

Title 6: Referral

Question 11 Enumerate five essential socio-demographic or clinical components that **MUST** be clearly documented inside a standard, structurally valid medical referral letter.

 Case Scenario:

Case 6: A family physician is caring for a patient with progressive symptoms of ischemic heart disease that are no longer responding efficiently to medical management at the primary care setting. He decides to transfer the care of this specific problem to a cardiologist.

Question 12 Clearly state the primary conceptual difference between a "Referral" and a "Consultation" within family medicine practice.
